

MEMORANDUM

To: Director of Risk Management, Mountain Vista School District
From: Niki Choksi, Risk & Claims Data Analysis
Date: August 15, 2025
Re: FY 2024-25 Workers' Compensation Claims Analysis - Findings & Recommendations

Summary

This memo summarizes an analysis of 375 workers' compensation claims filed across Mountain Vista School District's 10 sites during the 2024-25 school year (August 2024 – July 2025), totaling \$1,516,882 in incurred loss. Slip/fall incidents are the single largest cost driver - 31% of claims but 42% of total incurred loss - and cluster sharply in winter months at elementary campuses. Addressing winter slip/fall risk at elementary sites is the highest-leverage opportunity to reduce claims cost ahead of the 2025-26 school year.

Key Findings

- **Slip/Fall is the costliest cause of injury.** 117 claims (31% of all claims) totaled \$637,963 in incurred loss (42% of the district total). Among the 115 slip/fall claims with a known cost, the average claim runs \$5,548 - 35% above the district-wide average of \$4,111 (also computed over claims with a known cost; 6 district-wide and 2 slip/fall claims are still pending a loss estimate and are excluded from both averages).
- **Slip/fall claims are seasonal.** 75% of all slip/fall claims (88 of 117) occurred in the four winter months (November–February), consistent with icy/wet walkway conditions.
- **Winter slip/fall risk is concentrated at elementary schools.** 81% of winter slip/fall claims occurred at elementary sites, well above elementary schools' 51% share of claims district-wide - indicating elementary campuses carry disproportionate winter walkway risk, not just proportional volume.
- **Lifting/overexertion injuries carry a cost premium and are role-concentrated.** Lifting claims average \$5,670 (38% above the district average) and 77% occur in Facilities, Grounds, or Custodial roles - a clear target for a body-mechanics/safe-lifting training program.

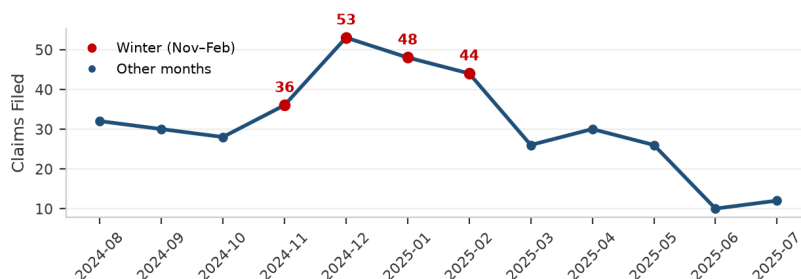


Exhibit: claims filed by month, Aug 2024–Jul 2025. Winter months (Nov–Feb) in red.

Recommended Actions

- Deploy an ice-melt / entry-mat / walkway-treatment program at elementary campuses, in place before November, ahead of the seasonal spike identified above.
- Launch targeted safe-lifting training for Facilities, Grounds, and Custodial staff, given the cost premium and departmental concentration of lifting claims.
- Add "date incident occurred" alongside "date reported" to the next RMIS export to enable reporting-lag tracking, which this export did not capture.
- Re-run this analysis quarterly using the existing Power Query pipeline to confirm the winter mitigation program reduces slip/fall incidence and cost in the 2025-26 season.

Data Quality Notes

The source RMIS export required substantial cleanup before analysis, documented in full in the accompanying cleaning log: 15 exact duplicate claim records were removed; 3 records with missing Claim IDs were retained under placeholder identifiers rather than deleted, per data-retention practice; report dates arrived in three inconsistent formats and were standardized, with 2 records carrying impossible reporting years quarantined for manual review; incurred loss values included currency-formatted text, negative entries, and outliers up to \$999,999 - 8 negative/outlier records were quarantined for adjuster confirmation rather than silently corrected; site, department, body part, cause, and status fields used inconsistent free text and abbreviations (e.g., "SA" for Student Assault, "O" for Open) and were standardized against a controlled-vocabulary mapping table; and employee type was missing on 12% of records, a source-data gap left blank rather than imputed.

Methodology & Privacy

This analysis uses fully synthetic data - no real students, employees, incidents, or protected health information are represented; Mountain Vista School District and all claim records are fictional constructs built for this project. In an operational context, claim-level records containing medical or personnel detail would be protected from disclosure under the Colorado Open Records Act's personnel/medical privacy exemptions; the aggregate, de-identified trend data presented here reflects the category of analysis appropriate for board or public reporting.